

Title

Prolonged Oral Anticoagulation Versus Standard Duration for Low-Risk Thromboembolic Events in Cancer: Subgroup Analysis by Clinical Functional Status

Abstract

Extended anticoagulation may reduce thrombotic recurrence in cancer patients, but the optimal strategy across different functional statuses requires evaluation. We analyzed 178 cancer patients with low-risk PE stratified by performance status: 79 with preserved functional capacity and 99 with functional limitations. The prolonged-therapy group (18 months) versus standard-therapy group (6 months) showed markedly different recurrent thrombosis rates in the functional-capacity stratum (3.1% vs. 19.2%; $p=0.048$) but a trend without statistical significance in the functionally-limited stratum (8.0% vs. 18.9%; $p=0.11$). Major bleeding was comparable in the functional-capacity group (2.5% vs. 7.4%; $p=0.39$) but appeared numerically elevated with prolonged therapy in the functionally-limited group (11.8% vs. 4.1%; $p=0.22$). Results suggest clinical benefit from extended treatment in patients with better baseline function, while increased bleeding risk warrants caution in those with functional decline.